Case study: Jayden

Condition: **Spinal cord injury – C6 AIS A tetraplegia**

19-year-old man · Inpatient spinal injuries rehabilitation unit, public hospital

Jayden Mitchell is a 19-year-old man with **C6 AIS A tetraplegia** following a diving injury when he dived into a creek with friends 10 weeks ago. He had surgical stabilisation of his cervical spine in the acute hospital and transferred to the spinal injuries rehabilitation unit four weeks ago. Before his injury, Jayden lived with his mother, Leanne, in a highset house in Ipswich and was in the second year of an electrical apprenticeship.

Jayden is described in the team notes as medically stable. He has a suprapubic catheter draining into a leg bag, wears an abdominal binder when sitting up, and his bowel programme is completed on alternate evenings. Physiotherapy has documented that he now tolerates sitting in his wheelchair for most of the day. Nursing notes from this morning record that he was "comfortable, no concerns", and his chart lists his usual blood pressure as around 100/65. Occupational therapy to date has focused on tenodesis training, wheelchair seating and feeding with a universal cuff.

You have been asked to complete an occupational therapy session in the unit's therapy room, setting up computer and gaming access and fitting a writing splint. Jayden arrives in good spirits. He uses his wrist extension well, tries a typing stick in a universal cuff, and tells you that getting back online with his gaming group is his "number one goal". He jokes that he will be "back to top of the leaderboard" within a week.

About twenty minutes into the session, Jayden stops and closes his eyes. He tells you he has a "pounding" headache, "like my head's going to explode". His face and neck have become flushed and there are beads of sweat on his forehead, although the room is cool. He keeps sniffing and says his nose has suddenly blocked up. When you ask him to read the screen, he says, "It's gone a bit blurry." Below the hem of his shorts, you notice goosebumps on his lower legs.

Jayden laughs it off. "I stayed up till three gaming with the boys. Serves me right." When you suggest stopping, he says, "Nah, it'll pass. Let's do the splint." As you move his wheelchair closer to the desk, you notice that the catheter tubing running to his leg bag is folded tightly where it has slipped under the edge of the wheelchair frame. The leg bag appears almost empty.

There is no documentation of Jayden's blood pressure or other observations since 8 am, and there is no clear record of whether he has had similar episodes before or when his leg bag was last emptied. You are not aware of the unit's specific procedure for this type of presentation, and there is no nurse in the therapy room.

While you are looking at the tubing, Leanne arrives early for a family meeting scheduled later in the afternoon. She is carrying a folder of NDIS access paperwork and photos of the stairs at the front of their house, and asks whether you have ten minutes to talk about ramps and a lift. Jayden's phone buzzes with a message from his mate Tyler, who is parking downstairs and coming up to visit. Jayden squints at the screen and says, "Mum, just give it a minute. It'll pass, it always does."

You are the occupational therapy student completing a computer access and splinting session with Jayden and your clinical supervisor is not currently in the room.

Present Jayden's case to your clinical supervisor using the ISBAR framework. Identify the immediate concerns and explain what you would prioritise in responding to the situation.

ISBAR planning sheet

Use your 10 minutes of preparation to plan. Then speak for 7 minutes to your “supervisor”.

I Introduction

≈ 20 s

Who you are, your role, who the client is and why you need to talk now.

S Situation

≈ 40 s

A brief but complete summary of what is happening and the immediate concern.

B Background

≈ 1 min

Relevant history: the primary problem, medical and social history, medications/allergies, interventions so far and OT to date. Name missing information rather than assuming it.

A Assessment

≈ 2 min 20 s

Your interpretation. Case finding → what it may mean → consequence for safety and occupations → priority. Separate facts, interpretation, gaps and proposed assessment. What needs escalation?

R Recommendation

≈ 2 min 40 s

What should happen next, in priority order. Action → reason → who → review. Immediate safety, escalation, MDT input, one short-term and one longer-term OT assessment, intervention, referrals and follow-up.

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